

SCOPE OF WORK

Going Merry Hospital Management System (HMS)

Codename: "Going Merry HMS"

Document Type	Scope of Work (SOW)
Prepared For	Sayantan Ghosh (Client / Business Owner)
Prepared By	Sarthak Kanade (Project Manager)
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Target Delivery Date	28 September 2026
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Date	05 September 2026
Status	Issued for Client Signature
Classification	Confidential

Binding Notice: This Statement of Work is a legally and contractually binding document once signed by both parties. It supersedes all prior verbal or informal discussions regarding project scope. It is to be read together with the approved BRD v4.0. In the event of ambiguity, this SOW governs delivery commitments, sprint sequencing, and acceptance; the BRD governs business intent.

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1. Purpose of this Document

This Scope of Work (SOW) defines the services, deliverables, timeline, responsibilities, and commercial boundaries for the design, development, and delivery of the Going Merry Hospital Management System (HMS) for Sayantan Ghosh (Client).

This SOW is derived from, and should be read together with, the approved Business Requirements Document (BRD) v4.0 (dated 29 August 2026), which governs detailed functional requirements and business intent. Where any conflict exists between this SOW and the BRD, this SOW governs commercial and delivery scope.

The System is a complete, non-MVP functional hospital management platform comprising 78 BRD-level business features, formally decomposed into 310 detailed delivery items across 16 functional modules. It will be delivered across 3 Sprints between 1 September 2026 and 28 September 2026 — Sprint 1 (1–6 September, foundation: planning, solution architecture, and roles & access setup), Sprint 2 (7–20 September, core module build), and Sprint 3 (21–28 September, hardening, testing, and go-live) — and deployed to Vercel. Microsoft Azure's free-tier access is valid for only 30 days, which does not cover the full project duration, so Vercel is used for application deployment instead.

2. Project Overview

The project involves building a complete, cloud-hosted hospital management platform covering the full operational journey: patient registration, appointment booking, check-in, live queue management, clinical consultation, diagnostics referral, prescription, pharmacy dispensing, inventory management, billing, notifications, reporting, and role-specific dashboards — secured by role-based access control and supported by advisory AI decision support.

Project Name	Going Merry Hospital Management System (HMS)
Client	Sayantan Ghosh
Engagement Type	Fixed-scope hospital management platform delivery
Target Delivery Date	28 September 2026
Primary Platforms	Web (responsive) — patient & staff interfaces
Deployment	Vercel (Development, Preview/UAT, Production environments). Note: Microsoft Azure free-tier access is valid for only 30 days, so Vercel is used for deployment to ensure uninterrupted environment availability across the full project.
Methodology	Agile Scrum — 3 fixed-scope, fixed-date Sprints: Sprint 1 (6 days), Sprint 2 (14 days), Sprint 3 (8 days)
Governing BRD	Hospital Management System BRD v4.0, dated 29 August 2026

3. Project Team & Roles

Project Manager	Sarthak Kanade
Tech Lead	Saurabh Yadav
Solution Architect	Ankit Verma

Business Analyst	Rizwan Salmani
Scrum Master	Sahil Shah
Client / Approving Authority	Sayantan Ghosh

4. Scope of Services

4.1 In-Scope Activities

- Requirements validation and solution design based on the approved BRD v4.0.
- Identity & Access Management (IAM): patient/staff login, registration, RBAC, session management, password recovery.
- Patient Management (PAT): patient master record/MRN, demographics, documents, search & duplicate handling.
- Appointment Management (APT): doctor/department discovery, slot availability, booking, rescheduling, cancellation, waitlist, reminders, and lifecycle management.
- Doctor Scheduling (SCH): recurring schedules, clinic sessions, leave/holiday management, slot/capacity/room configuration, conflict detection.
- Smart Queue Management (QUE): patient check-in, token generation, live queue display, priority & triage rules, patient call/recall, queue transfer, pause, cancellation, and fallback handling.
- Clinical Encounter / EMR (EMR): encounter creation, triage & vitals, clinical notes, problem list, diagnosis, orders, prescriptions, follow-up, and history.
- Diagnostics — Laboratory & Radiology (DIA): test catalog, diagnostic orders, specimen/work queue, result entry & verification, critical-result alerts, and patient report viewing.
- Pharmacy & Medication Dispensing (PHA): prescription queue, pharmacist validation, drug catalog, interaction/allergy warnings, dispensing (full/partial), dispensing history, refills, and substitution.
- Inventory & Procurement (INV): item master, batch/expiry tracking, stock ledger, inter-location transfer, low-stock alerts, supplier master, purchase orders, and goods receipt.
- Billing & Payments (BIL): service/charge catalog, charge capture, invoice generation, discounts/tax/waivers, payment collection, refunds, reversals, receipts, and reconciliation.
- Inpatient / Ward Management (IPD): admission request/approval, bed/ward/occupancy management, patient movement, inpatient care coordination, discharge summary, and final billing.
- Notifications & Communications (NOT): channel preferences, consent, templates, appointment/queue/prescription notifications, delivery tracking, retry, and communication history.
- Dashboards, Reporting & Analytics (REP): patient, doctor, pharmacy/inventory, admin, and management KPI dashboards; trend reports; scheduled report generation; export.
- Administration & Configuration (ADM): hospital/department/master-data setup, user lifecycle, business rules, system configuration, and integration settings.

- AI Decision Support (AI — advisory only): waiting-time prediction, no-show prediction, queue optimization recommendations, inventory demand forecasting. AI is advisory; all workflows retain deterministic non-AI fallback paths.
- Security, Privacy & Compliance (SEC): RBAC/least-privilege, PII masking/tokenization, encryption in transit and at rest, audit logging, backup/restore, and controlled emergency access.
- Cloud deployment on Vercel with separate Development, Preview/UAT, and Production environments; CI/CD pipeline; monitoring; backup and DR procedures.
- Quality assurance: unit, API/integration, system, security, performance, AI, and regression testing prior to each Sprint sign-off.
- Documentation handover: BRD, SOW, technical design, traceability matrix, and admin/user guide.
- Post-delivery warranty support — 30 days from go-live — for defects introduced during development (bug-fix only, not new features).

4.2 Out-of-Scope Activities

- Laboratory machine / instrument integration; LIS/RIS systems; PACS imaging archive; specimen logistics/courier systems.
- Insurance claims processing; TPA integration; pre-authorization workflows; claim adjudication; external accounting / GL systems (Tally, SAP, QuickBooks, etc.).
- ICU management; OT scheduling; NICU management; nurse rostering; ambulance dispatch; housekeeping workflows; medical equipment asset management; meal planning; complex multi-department discharge orchestration.
- HRMS, payroll processing, recruitment / applicant tracking, biometric attendance systems.
- Automated integration with external supplier ERP systems or EDI procurement automation.
- Autonomous clinical diagnosis, autonomous prescribing, or autonomous treatment decision-making of any kind. AI in this System is advisory decision-support only and will never replace clinician or pharmacist judgment.
- Integration with any external medical device or diagnostic hardware unless explicitly approved via Change Request.
- Selection and integration of a specific payment gateway / provider (this SOW covers the payment-status and transaction data model, not a bound gateway integration).
- Integration with any specific third-party messaging platform (e.g., WhatsApp Business API, named SMS gateway) beyond a generic provider-agnostic notification abstraction layer.
- Native iOS/Android app builds — the patient/staff experience is delivered as a responsive web / mobile-web interface.
- Multi-hospital, multi-branch, or multi-tenant operation — scoped for single-facility deployment.
- Multi-language / internationalization support beyond English.
- Ongoing infrastructure cost (Vercel usage/consumption billing, and any third-party API/service fees) beyond the project delivery window.
- Formal third-party compliance certification (HIPAA, ISO 27001, SOC 2 audit).

5. Deliverables

#	Deliverable	Format
1	Identity, Access & Account Management (IAM)	Deployed module
2	Patient Management (PAT)	Deployed module
3	Appointment Management (APT) — Primary Scope	Deployed module
4	Doctor Scheduling (SCH)	Deployed module
5	Smart Queue Management (QUE) — Primary Scope	Deployed module
6	Clinical Encounter / EMR (EMR)	Deployed module
7	Diagnostics — Laboratory & Radiology (DIA)	Deployed module
8	Pharmacy & Medication Dispensing (PHA) — Primary Scope	Deployed module
9	Inventory & Procurement (INV) — Primary Scope	Deployed module
10	Billing & Payments (BIL)	Deployed module
11	Inpatient / Ward Management (IPD)	Deployed module
12	Notifications & Communications (NOT)	Deployed module
13	Dashboards, Reporting & Analytics (REP)	Deployed module
14	Administration & Configuration (ADM)	Deployed module
15	AI Decision Support (AI)	Deployed module
16	Security, Privacy & Compliance (SEC)	Deployed module
17	Vercel Deployment Environments (Dev, Preview/UAT, Production)	Vercel deployment
18	Technical Design Document	.docx / .pdf
19	Test Summary Report (QA sign-off)	Report
20	Admin & User Guide	Document
21	Source Code & Deployment Access	Git repository handover

6. Technology & Deployment

Note: Microsoft Azure provides free-tier access for only 30 days, which does not cover the full project timeline. To ensure Development, Preview/UAT, and Production environments remain available and uninterrupted throughout the engagement, the System is deployed on Vercel instead of Microsoft Azure.

- Backend & Database: Vercel (serverless functions / API routes) for application hosting; a managed cloud Postgres database (e.g., Vercel Postgres or an equivalent managed provider) for transactional records; Vercel Blob Storage for documents, reports, and bill images; platform-managed environment variables for secrets management; a managed queue/event service for asynchronous notification events.

- Application Architecture: Modular monolithic with clearly bounded internal domains (Appointment, Queue, Pharmacy, Inventory, Clinical, Billing, Platform). Separate deployable services introduced only where there is a documented justification.
- AI Engine: A managed AI/ML API (or equivalent) for waiting-time prediction, no-show prediction, queue optimization, and inventory demand forecasting — advisory only, with deterministic rule-based fallback mandatory for every AI-assisted workflow.
- Security: TLS/HTTPS enforced in transit; AES-256 encryption at rest for the primary database and object storage; RBAC with least-privilege defaults; PII masking for role-inappropriate views; platform-native audit logging and security monitoring.
- Client Interfaces: Responsive web / mobile-web interfaces for Patient, Doctor, Nurse, Pharmacist, Inventory/Procurement, Billing, Administrator, and Management roles.
- Deployment Environments: Vercel hosts three environments — Development (Delivery Team only), Preview/UAT (Delivery Team + named Client reviewers), and Production (live operational use after UAT sign-off).
- CI/CD Pipeline: Repeatable, documented deployment and environment configuration via Vercel's Git-integrated CI/CD; no one-time manual processes.
- Monitoring & Observability: Application health, error rates, queue depth, integration status, and security events observable through Vercel-native monitoring/logging tooling (and provider-native monitoring for the managed database).
- Backup & DR: Backup/restore procedures defined, implemented, and tested at least once before Production sign-off, with documented RPO and RTO.

7. Project Timeline & Milestones

The project is executed over 3 Sprints totalling 28 days, starting 1 September 2026 and targeting go-live on or before 28 September 2026. Sprint 1 is a short foundation sprint (planning, architecture, and access setup); Sprint 2 is the largest sprint, covering the bulk of module build-out; Sprint 3 closes out the project with integration testing, hardening, and go-live.

Phase	Duration	Dates	Key Focus
Sprint 1	6 days	1–6 Sept 2026	Planning & SOW/architecture finalization, Vercel environment setup, RBAC roles & access foundation, project scaffolding
Sprint 2	14 days	7–20 Sept 2026	Patient Management, Scheduling, Appointments, Smart Queue, Clinical Encounter/EMR, Notifications, Diagnostics, Pharmacy, Inventory, Billing, IPD, Dashboards & Reporting, Administration, AI Decision Support, Security build-out
Sprint 3	8 days	21–28 Sept 2026	Integration & regression testing, security/performance hardening, Vercel production deployment, UAT sign-off, documentation handover, go-live

Sprint 1 (1–6 September 2026)

Sprint Goal: Finalize planning and solution architecture, stand up the Vercel environments, and establish the roles & access (RBAC) foundation that every later module depends on.

Priority: High — Foundation Critical Path

Module	Features Delivered	Story Points
PLN — Planning & Solution Architecture	SOW/BRD finalization & sign-off; solution architecture & module boundary design; technology stack finalization; database schema design	10
ENV — Environment & DevOps Setup	Vercel project & repository setup; CI/CD pipeline skeleton; Development and Preview environment provisioning	8
IAM — Roles & Access Foundation	RBAC role & permission matrix definition; authentication scaffold (login/registration skeleton); session management foundation	14
GOV — Project Governance	Sprint planning & backlog grooming; RACI and communication setup	6
	Sprint 1 Total	38

Sprint 1 End-of-Sprint Deliverable (Demonstrable to Client): Approved SOW/architecture baseline; Vercel Development and Preview environments provisioned; RBAC role/permission matrix defined; authentication scaffold running end-to-end (login screen wired to a backend stub).

Sprint 2 (7–20 September 2026)

Sprint Goal: Build out the complete set of functional modules — the full patient-to-billing journey, supporting modules, and AI decision support — on top of the Sprint 1 foundation.

Priority: Critical — Primary Scope + Full Module Build-Out

Module	Features Delivered	Story Points
PAT — Patient Management	Patient record & MRN; demographics & contacts; patient search & duplicate handling; documents & record access	16
SCH — Doctor Scheduling & Capacity	Doctor schedule & clinic sessions; leave/holiday management; slot duration, capacity & rooms; schedule exceptions & conflict detection	18
APT — Appointments & Referrals (Primary)	Doctor/department discovery; availability & slot search; appointment booking; rescheduling & cancellation; waitlist & referral; lifecycle, history & reminders	26
QUE — Check-in, Token & Queue Management (Primary)	Patient check-in & eligibility; token generation & queue entry; live queue display & patient position; priority/triage rules; patient call, recall & turn management; queue transfer, pause, cancellation & fallback	39
NOT — Notifications & Communications	Notification preferences & templates; appointment/queue/doctor-delay notifications; delivery tracking & retry	16
EMR — Clinical Encounter / EMR	Encounter creation & patient context; triage, vitals & symptoms; clinical notes & problem list; medical/medication history; diagnosis, orders & prescription creation; encounter signing & follow-up	34

Module	Features Delivered	Story Points
PHA — Pharmacy & Medication Dispensing (Primary)	Prescription queue & pharmacist validation; medicine/drug catalog; medication interaction & allergy warnings; dispensing (full/partial); dispensing history, refills & substitution	29
INV — Inventory & Procurement (Primary)	Item master & location management; batch, expiry & stock status; stock ledger, adjustments & reconciliation; inter-location transfer; low-stock thresholds & alerts; reorder requests, suppliers, POs & goods receipt	34
DIA — Diagnostics — Laboratory & Radiology	Test catalog; diagnostic order & scheduling; specimen/work queue; result entry & verification; diagnostic reports & critical-result alerts	18
BIL — Billing & Payments	Service & charge catalog; charge capture & invoice generation; discount/tax/waiver rules; payment collection; refunds & reversals; receipts, statements & reconciliation	22
IPD — Inpatient / Ward Management	Admission request & approval; bed/ward/occupancy management; patient admission, transfer & movement; inpatient care & discharge coordination; discharge summary & final billing	19
REP — Dashboards, Reporting & Analytics	Patient dashboard; doctor & clinical dashboard; pharmacy & inventory dashboard; admin & operational dashboard; management KPI, trend & scheduled reporting	28
ADM — Administration & Configuration	Hospital/department/master-data configuration; user lifecycle & staff onboarding; business rules & configurable workflows; system configuration & integration settings	22
AI — AI Decision Support	Waiting-time prediction (AI-01); no-show prediction & appointment optimization (AI-02); queue optimization recommendations (AI-03); inventory demand forecasting & replenishment insight (AI-04)	39
SEC — Security, Privacy & Compliance	RBAC enforcement & least privilege; PII masking/tokenization; encryption, audit logging & security monitoring; backup, restore, retention & emergency access	25
	Sprint 2 Total	385

Sprint 2 End-of-Sprint Deliverable (Demonstrable to Client): A fully integrated, demonstrable Hospital Management System deployed to the Vercel Preview/UAT environment, covering the complete end-to-end business journey — Patient Registration → Login → Find Doctor → Book Appointment → Check-in → Queue → Consultation → Diagnostics → Prescription → Pharmacy Dispensing → Inventory Deduction → Billing → Dashboards — with decision-support AI layered on operational data and security controls in place.

Sprint 3 (21–28 September 2026)

Sprint Goal: Close out the project — integration and regression testing, security and performance hardening, production deployment on Vercel, UAT sign-off, documentation handover, and go-live.

Priority: Critical — Testing, Hardening & Go-Live

Module	Features Delivered	Story Points
QA — Integration & Regression Testing	End-to-end regression testing; API/integration testing; security testing; performance testing; AI validation testing	20
DEP — Production Deployment & Cutover	Vercel production environment setup; environment variable & secrets migration; domain/DNS cutover; monitoring & alerting setup	12
UAT — UAT Support & Sign-off	UAT session support; defect triage & fixes; UAT sign-off documentation	10
DOC — Documentation & Handover	Technical Design Document; Admin & User Guide; Test Summary Report; source code & deployment access handover	8
	Sprint 3 Total	50

Sprint 3 End-of-Sprint Deliverable (Demonstrable to Client): A fully tested and hardened Hospital Management System deployed to Production on Vercel, with Client UAT sign-off obtained, all documentation handed over, and the 30-day post-go-live warranty period beginning.

Note: This is an indicative plan assuming a 1 September 2026 kick-off. Any delay in kick-off, Client-side approvals, or Client dependency fulfillment (Section 9) will shift dates accordingly on a day-for-day basis.

8. Acceptance Criteria

- All 78 BRD-level features / 310 SOW/SRS delivery items (across all 16 modules) are implemented and demonstrable.
- The four Primary Scope pillars — Appointment Management, Smart Queue Management, Pharmacy Queue Management, and Inventory Management — pass full acceptance testing before Sprint 3 sign-off, with no unresolved Critical or High severity defects.
- The complete end-to-end patient journey (registration → appointment → check-in → queue → consultation → prescription → pharmacy → billing) is demonstrable in the Vercel Preview/UAT environment.
- AI Decision Support outputs display correctly with confidence/context indicators; deterministic non-AI fallback paths activate correctly when AI is unavailable.
- AI is strictly advisory — no autonomous clinical diagnosis, prescribing, or treatment decision-making is implemented.
- RBAC is verified for all roles; unauthorized actions are rejected server-side; PII masking is verified for at least one sensitive field type in non-privileged views/logs.
- Encryption in transit (TLS/HTTPS) and at rest (primary database and object storage) is confirmed.
- Backup/restore procedures are executed and validated at least once before Production sign-off, with documented RPO and RTO.
- Concurrency protections are verified: appointment booking, queue/token operations, and inventory dispensing prevent conflicting concurrent updates.
- The application passes agreed QA test cases with no open Critical or High severity defects at UAT sign-off.
- Client provides written (email or signed document) UAT sign-off prior to production go-live.

9. Assumptions & Dependencies

- Client will provide timely access to the Vercel project/account (or approve the Delivery Team provisioning one) with sufficient permissions to configure required resources. Delay in this access extends the delivery calendar day-for-day.
- Client will provide a domain name, or explicitly approve use of a Delivery-Team-provided interim subdomain.
- Client will provide any third-party credentials required for agreed integration adapters (e.g., selected payment provider, SMS/email sender) in a timely manner.
- Client will nominate a single primary point of contact with authority to approve UAT sign-off, scope clarifications, and Change Requests, to avoid conflicting instructions.
- Client will make the primary point of contact (or a named delegate) available for UAT review within 2 business days of each Sprint's Preview/UAT deployment.
- Any delay in Client-side approvals, content/asset delivery, or third-party account provisioning will proportionally shift the 28 September 2026 delivery date.
- The project uses project-controlled sample/test data throughout Development and Preview/UAT unless real production data is explicitly authorized in writing by the Client, with an accompanying data-protection agreement.
- No specific third-party integration (payment gateway, SMS/email provider, insurance system, laboratory system, etc.) is assumed until a specific provider is selected and formally approved via Change Request.
- AI model quality depends on sufficient historical/operational data; the rule-based fallback remains the primary behavior during the initial post-deployment period until sufficient data accumulates. This is not a defect.
- Ongoing Vercel usage/consumption cost, and any third-party API/service fees, beyond the delivery window are Client-borne.
- Any new feature request beyond the BRD-defined scope will be handled through the Change Control Process (Section 11).

10. Roles & Responsibilities Matrix (RACI)

R = Responsible, A = Accountable, C = Consulted, I = Informed

Activity	PM (S. Kanade)	BA (R. Salmani)	Scrum Master (S. Shah)	Tech Lead (S. Yadav)	Solution Architect (A. Verma)	Client (S. Ghosh)
Requirements Sign-off	A	R	I	C	C	A
Architecture & Tech Decisions	I	C	I	C	A/R	I
Sprint Planning & Tracking	A	I	R	C	I	I
Development	I	I	C	R	C	I
Vercel Deployment	A	I	I	R	A/R	I

Activity	PM (S. Kanade)	BA (R. Salmani)	Scrum Master (S. Shah)	Tech Lead (S. Yadav)	Solution Architect (A. Verma)	Client (S. Ghosh)
QA & Testing	C	C	C	A/R	C	I
UAT Sign-off	A	C	I	C	C	R
Production Go-Live Decision	A	I	I	R	C	A
Change Requests	A	R	C	C	C	A

11. Change Control Process

- After this SOW is signed, the scope defined herein is fixed and can only be changed through the formal Change Request procedure below. No email, chat message, verbal instruction, meeting comment, or informal Sprint-demo feedback constitutes an approved scope change.
- Any request that adds, removes, or materially alters functionality beyond the approved BRD/SOW scope will be logged as a Change Request (CR) by the Business Analyst.
- The Business Analyst and Solution Architect will jointly assess impact on timeline, effort, and cost within 3 business days of CR submission.
- The Project Manager will present the impact assessment to the Client for written approval before any change is implemented.
- Approved CRs affecting the 28 September 2026 delivery date will result in a mutually agreed, documented date revision. An approved CR does not compress into the existing calendar at the expense of Primary Scope delivery.
- No out-of-scope work will begin without written (email or signed) Client approval of the associated CR.
- The following are non-negotiable scope boundaries that no ordinary CR can override and require full SOW renegotiation: (a) Autonomous clinical diagnosis, prescribing, or treatment decision-making by AI; (b) any integration requiring real patient PII/PHI before appropriate data-protection arrangements are confirmed; (c) compression of the 28-day calendar to accommodate added scope without a corresponding schedule and/or commercial adjustment.

12. Data, Security & Compliance Commitments

- All data will be encrypted in transit (TLS/HTTPS, enforced for all client-facing and inter-service traffic) and at rest (AES-256 encryption for the primary database and object/file storage).
- Sensitive fields (PII, patient identifiers, clinical data) will be masked in role-inappropriate views/logs, with users seeing only the information necessary for their role and current business purpose.
- Credentials and secrets are stored exclusively as managed environment variables within Vercel; no hard-coded credentials appear in source, configuration files, or client-visible build artifacts.
- RBAC with least-privilege defaults: every protected action is authorized server-side using role and, where applicable, department/record-level scope.

- All sensitive create/update/delete/approve/access actions on patient, clinical, prescription, inventory, and billing data leave an auditable history of actor, action, and timestamp.
- Test and Development environments do not expose unnecessary real PII; project-controlled sample/test data is used unless real production data is explicitly authorized in writing by the Client.
- Backup and restore procedures are defined, implemented, and tested at least once before Production sign-off, with a documented Recovery Point Objective (RPO) and Recovery Time Objective (RTO).
- The System is built with security-conscious practices aligned with HIPAA, ISO 27001, and DPDP Act principles; formal third-party compliance certification is a distinct, separately scoped engagement if required.

13. Sign-off

This Scope of Work is issued for acceptance in the manner of a standard terms-and-conditions agreement: by accepting/signing off on this document (including electronic or online acceptance), the Client agrees to the scope, deliverables, timeline, and terms described herein, and confirms it as the complete, binding scope baseline for the Going Merry Hospital Management System engagement.

Acceptance is expected to be completed online. From the point of acceptance forward, the scope defined herein is fixed and any change is governed exclusively by Section 11.